



Managing Chronic Constipation and Soiling in Children

Information booklet for parents/carers



Government
of South Australia

Health

Southern Adelaide
Local Health Network

Information booklet for parents/carers

This booklet has been developed by staff on the Paediatric Unit at Flinders Medical Centre to assist you with your child's constipation. The contents of this booklet will be discussed with you. Please do not hesitate to ask if you are unsure of anything in this booklet.

Acknowledgments

Noha Soliman - Consultant Paediatrician
Rachel Wilson and Claire Wilson - Clinical Nurses
Rosina Gergis and Luke Grzeskowiak - Senior Pharmacists
Debbie Atkins and Melanie Tripptree - Occupational Therapists
Charlotte Manning - Clinical Dietitian

Contents

Managing Chronic Constipation and Soiling in Children 4

Medical Management 5

Medications 6

Choose your Poo chart 9

Diet and Fluids 10

Exercise/Toilet set up/routine/posture 12

Diary 14

Resources 17

References 18

Appendices 19

This book belongs to

.....

Managing Chronic Constipation and Soiling in Children

The Poo Challenge

Learning to use the toilet takes time. Many children still accidentally wet or soil their pants a year or more after training begins. Unfortunately for some children, the road to bowel continence is complicated by constipation.

This booklet aims to give parents and carers important tips to help their child be in charge of their poos! Please ask the health worker who gave you this booklet if you have any questions.

Constipation and Soiling

What is Constipation?

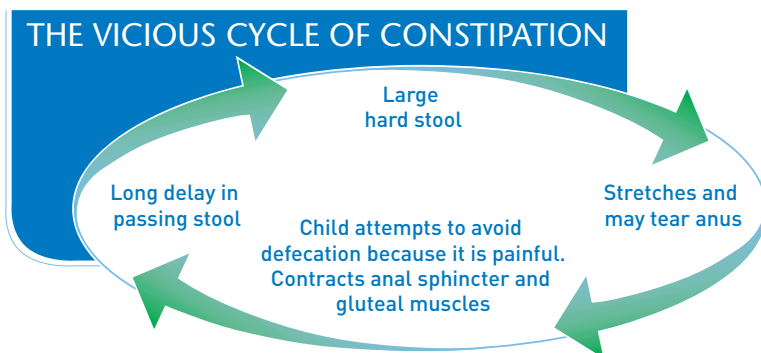
Constipation is a very common condition that affects approximately 5-30% of children. Constipation in children is identified by two or more of the following symptoms in the last 4 weeks for all children:

- Hard stools (may be pebble-like poo) for most poos and/or pain while passing poo
- Two or less bowel movements a week
- Passing poos so large they block the toilet
- Withholding postures
- At least one poo 'accident' (soiling) per week.

Constipation often results in excessive straining and increased time spent on the toilet, incomplete poos and soiling.

The most common cause of constipation in children is the withholding of stool following some trigger event, which might be:

- A single episode of painful poo
- Commencing toilet training
- Disruption in routine (travel, new sibling)
- Illness/some medications
- New/stressful situations (starting school/ kindy – especially if no privacy)
- Reduced fluid intake or diet



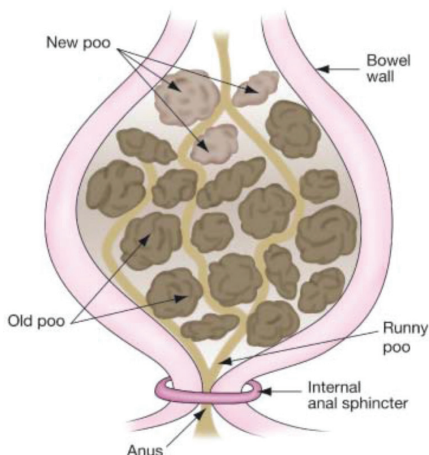
Why Does My Child Soil?

Soiling (also known as faecal incontinence) is when poo is inappropriately passed (e.g. into undies or onto the floor). By far the greatest reason for soiling is constipation. Of children who soil, 95% also have chronic constipation.

When a child has been constipated for a long time, the amount of poo inside their body builds up until the rectal (bowel) wall becomes stretched. A stretched bowel wall does not give them a clear message that it needs to be emptied, and by the time the child is aware something's coming, it may be too late!

Also, when the bowel is full of hard, dry (impacted) stool, newer liquid stool can leak out around the blockage into the child's underwear without the child even knowing. (Sneaky poo)

Soiling is NOT deliberate, or because of laziness. Instead, the constipated bowel is not giving the signals that the child needs to get to the toilet on time.



It is important to have your child under the care of a doctor (your GP or Paediatrician) who knows about childhood constipation and soiling problems. Your doctor will feel your child's tummy and may order an x-ray. They will talk to you about medications, toileting routines and diet.

Medical Management

There are three main steps to managing constipation:

STEP 1: CLEAR OUT the blockage of poo. Your doctor may suggest:

- Medicines (see Medicine section below)
- A visit to hospital for a few days to have a 'washout' (See below)

STEP 2: MAINTENANCE of soft poos and regular toilet time

- After the Clear Out, poo must be kept very soft – just like cow poo, porridge or wet cement. This needs to be maintained for at least 6 months after successful treatment of the constipation and/or soiling. This is so that the bowel can recover to its normal size and function and so that pooing NEVER hurts. Your doctor will recommend the best medication maintenance dose to achieve this.

A regular toileting program will also help to maintain healthy bowels. Your health worker can help you with this. It is important to note that a stretched bowel can take at least 12 months to recover, so be prepared for the long road ahead.



STEP 3: RESCUE PLAN

Relapses are common, and the faster you notice the symptoms and respond, the better. Discuss with your doctor a rescue dose of medication that you can give your child if they have not done a full poo in the last 48 hours. Then get to the doctor soon afterwards for follow-up.

Your child might do poo every day but has other signs that suggest relapse including: pain and straining to do poos, pooing extra large diameter poo, or incomplete emptying of the bowel by pooing very small amount many times during the day.

Medications

Children who have been constipated for many months are likely to need laxative medications for several months in addition to encouraging healthy bowel habits. Many medications are available for managing constipation in children and are usually taken as a liquid (some products come as a powder that needs to be mixed with liquid first).

Consult with your child's doctor for the appropriate medication for your child.

Below is a summary of laxatives used in children with some tips:

Prune juice - this is a mild, natural laxative that works in some children. Prune juice may taste better if mixed with another juice, such as apple, apricot or cranberry juice. You can freeze prune juice to make icy poles.

Natural fibre supplement (e.g. Fybogel®, Metamucil®) helps make poo bulky but soft and easy to pass out. It comes in both unflavoured and orange flavour. Do not give immediately before going to bed. It is important to ensure your child drinks fluids when taking this. It can cause smelly wind (farting) and bloating.

Liquid paraffin mixtures - (e.g. Agarol®, Parachoc®) come as a flavoured liquid, and they work by softening and lubricating the poo to make it easier to pass. This liquid should not be given at night or immediately before lying down as it may cause aspiration (Paraffin oil being regurgitated and inhaled into the lungs).

Macrogol 3350 - (e.g. Movicol®, Movicol Junior®, OsmoLax®, Macrovic®) comes in a sachet to mix with water and works by softening the poo. It can cause tummy cramps.

Lactulose - (e.g. Duphalac®, Actilax®) comes as a sweet-tasting liquid, and works by softening the poo and stimulating the bowel to empty. It may taste better mixed with juice, water or milk. It can cause smelly wind (farting).

Docusate / Poloxamer - comes as a tablet (e.g. Coloxyl®) or drops (Coloxyl Drops®- most suitable for children under three years of age), and works by softening the poo. The drops can be mixed with formula or fruit juice. It is very rare but your child may have tummy cramps.

Senna - (e.g. Laxettes with Senna®, Senna-G®, Senokot®), comes as a tablet, and works by stimulating the bowel to empty. Your child may have diarrhoea or stomach cramps if the dose is too high. Giving Senokot® at night may reduce tummy cramping.

Bisacodyl - (e.g. Bisalax tablets®) works by stimulating the bowel to empty. It can cause tummy cramps. Giving this at night may reduce tummy cramps.

Suppositories & mini-enemas - (e.g. Microlax®, Glycerol Infant®, Glycerol Child®, Dulcolax® suppositories) these are small tablets or liquid that is placed into your child's bottom (rectum). They stimulate the rectum to empty. They do not soften the poo in the upper bowel. They are sometimes recommended for severe constipation but oral (swallowed) laxatives are more effective and less distressing for most children. Glycerine suppositories (Glycerol infant®) may be used for severe constipation in infants.

Never give an enema to your child unless your child's doctor has told you to and you have been instructed on how to give it.

Bowel irrigation - a very small number of children get so severely constipated that they need admission to hospital for a bowel washout. This is usually done using several litres of 'bowel prep' fluid. The fluid can be given as a drink or put down a tube into the stomach (nasogastric tube).

Monitor your child's progress with medication

To help prevent recurrence of constipation, laxative treatment is usually continued for several months. The following goals must be met by the therapy and they should include:

1. Regular daily or more than daily soft non painful bowel movements
2. No soiling
3. Elimination of stool withholding behaviour

Once the goals of maintenance therapy have been achieved it will need to be continued for at least 3-6 months for long term symptom resolution and there will need to be ongoing medical review and re-assessment over this period.

Do not stop the laxatives suddenly. Stopping laxatives suddenly might cause the constipation to quickly return. Your doctor will normally advise a gradual reduction in the dose over a period of time, depending on how the stools have become in their consistency and frequency.

Chronic constipation with or without soiling is a chronic condition which can take months to years to fully control. It is very important not to stop your child's medication too soon or suddenly to make sure the bowel has time to fully recover.

What if your child won't take the medication?

Remind your child why the medication is important. Try a small reward, for example give them a sticker or a special drink after the medicine. Other ideas include:

- Drink it really cold
- Use a favourite cup/bottle just for medicine
- Try a straw which helps to avoid some taste buds
- Some medicines can be mixed with other drinks (as mentioned above), some have flavour and some no flavour at all, so ask your doctor or pharmacist about more palatable options.

Maintaining a Healthy Bowel for Life!

Learning to take care of our bowel is important for everyone's lifelong health! Chronic constipation is a long-term condition that needs ongoing management by the parent and child, with the child increasing in independence as they get older.

Monitor Healthy Bowel Patterns

The first key to maintaining healthy poos is by checking the poos regularly. The Choose Your Poo chart is a helpful guide for checking the composition of the poos. Poo that is runny or dry, frequent or infrequent indicates an unhealthy bowel pattern. So, it is important to teach your child to look before they flush.

Use the Choose Your Poo Chart scale to help your child score their poos! This can aid in early detection of constipation. Healthy poos are levels 4 - 5. A child recovering from constipation needs to have their poos kept at level 5 - 6 for 6 months after treatment success.

Learning to take care of our bowel is important for everyone's lifelong health! Children especially need to be taught and encouraged to listen and respond to their body, as well as actively make good choices about what they put into it and how they move it.

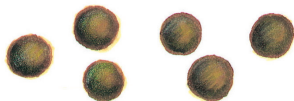


THE BRISTOL STOOL FORM SCALE (for children)

choose your

Poo!

type **1**



looks like:

rabbit droppings

Separate hard lumps, like nuts (hard to pass)

type **2**



looks like:

bunch of grapes

Sausage-shaped but lumpy

type **3**



looks like:

corn on cob

Like a sausage but with cracks on its surface

type **4**



looks like:

sausage

Like a sausage or snake, smooth and soft

type **5**

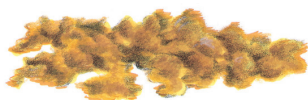


looks like:

chicken nuggets

Soft blobs with clear-cut edges (passed easily)

type **6**



looks like:

porridge

Fluffy pieces with ragged edges, a mushy stool

type **7**



looks like:

gravy

Watery, no solid pieces ENTIRELY LIQUID

Diet and Fluids

Fluid intake

The second key to regular, healthy poos is adequate daily fluid intake. It is essential that your child drinks plenty of fluid throughout each day:

School age children need to be drinking 4-6 cups of fluid per day, and possibly more in hot weather. Remember that most cups are about 200ml.

It is best to drink full drinks a number of times during the day, rather than constantly sipping small amounts.

What if your child won't drink enough fluid?

- Fill up a large water bottle for your child; encourage them to finish it twice during the day.
- Try a transparent water bottle for school with lines down the side at 150-200 mls intervals as guide (to encourage full drink not just a sip)
- Take 2 bottles of water to school and have your child keep one on their desk, and one with their lunchbox. Discourage use of water fountains as they encourage little sips and can't be measured
- Add a squeeze of lemon or lime (use with caution to prevent erosion to the enamel on the teeth), or a sprig of mint to the water to enhance the flavour
- Add a drop of food colouring – what colour will it be today?
- Encourage the whole family to drink a whole glass of water at each meal and snack time



Water is the best drink for your child. Small amounts of diluted juice or sugar free cordial can be used. Pear juice and prune juice contain natural stool softeners.

It is best to avoid cordials containing sugar and to avoid soft drinks. It is also best to avoid offering an excessive amount (more than 2 cups/day) of cow milk to your child.

All these will reduce your child's appetite for a more nutritious balanced diet leading to iron and vitamins deficiencies, and your child will not get enough fibre in their diet which could make constipation worse.

A Healthy Diet

A third key to maintaining bowel health is a nourishing diet with adequate daily fibre intake. This helps keep the bowel regular. However eating too much fibre can worsen symptoms especially if your child is not drinking enough fluid. It is important to consume enough fibre and liquid that will help your child bowel to work regularly.

As a general rule of thumb, the less processed a food item, the higher the fibre content. This can be done by offering your child fruits and vegetables, beans and legumes, as well as choosing whole grain or wholemeal breads, cereals and crackers.

How much fibre do children need?

The following is a guide of the suggested amounts of fibre children of all ages need.

1-3 yrs – 14 grams per day

4-8 yrs – 18 grams per day

9-13 yrs –

Boys: 24 grams per day

Girls: 20 grams per day

14-18 yrs –

Boys: 28 grams per day

Girls: 22 grams per day

See tables in appendix 1 for general guidance.

If you have any questions, please talk to your child's dietitian.

Tips for Increasing Fibre:

- Make fresh fruit & vegetable snacks available, including the skin where possible (e.g. leave a fruit bowl out)
- Include legumes and beans in your cooking (e.g. beans, lentils, dried peas in casseroles, soups or mince)
- Add fruit, vegetables and bran products in your cooking by grating/adding them into dishes such as mince, rissoles, and spaghetti bolognaise. Try carrots, apple, pear and zucchini.

Exercise/Toilet set up/ routine/posture

Play and Movement

Regular physical activity promotes core strength, body awareness, can help reduce stress and promotes a sense of wellbeing.

Encourage your child to move their body during playtime at school/kindy and at home. You will probably have to limit 'screen' time (TV, computer, console games) and involve the whole family!

What are some physical activities your child enjoys that you could encourage him/her to do more regularly? Are there some errands outside of the home that your child could do? (e.g. walk to the corner store, take the letters to the post box).

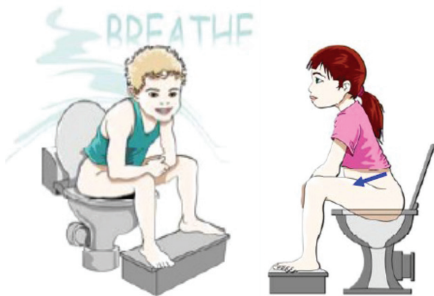
Toilet Set-Up

Your toilet set up needs to suit your child so that he/she feels comfortable, relaxed and secure while sitting on the toilet.

Some children become fearful of using the toilet because they are worried they might fall in, they don't like the sensation of splashes, or they are scared of the pain they have felt in the past.

Relaxing is crucial to successful pooing! Here are some important things to have in place in your toilet.

- A child-sized toilet seat/insert so they don't feel like they are in danger of falling in! An insecure position contributes to tensing the bottom muscles rather than letting poo come out. Look around for an insert that suits your child's size. A double family toilet seat (available from Hardware stores) is a good option to consider.
 - A secure foot-stool to use while sitting on the toilet provides stability so your child can get into the best pooing position and relax. The foot stool should be high enough so that their feet are supported with knees positioned slightly higher than their hips. This is the best body position to help the poo come out.
 - Check the toilet room together with your child:
 - Does it smell? Get an air freshener.
 - Is it dark? Put in a brighter globe or lighten the window coverings.
 - Are there cobwebs? Time to remove them!
 - Maybe it's just boring. Put a few favourite pictures, photos, a calendar or other decorations on the walls – try making some together!
- Remember – it should be a relaxing, inviting place that your child feels comfortable in.
- Toilet Paper should be within easy reach, soft and easy to use. If your child struggles with wiping until they're clean, try flushable wet-wipes. And make sure extra rolls are always handy.



The best way to do a poo

1. Bottom at the back of the toilet seat, feet resting on a foot stool so that knees are apart and slightly higher than the hips
2. With a straight back, lean forward so the hips are bent
3. Relax by doing some slow, deep breathing down into the tummy
4. Let the tummy bulge forward between the thighs
5. Let the 'gate' open gently. Keep breathing slowly and gently, waiting
6. Think about how it feels inside the bottom. Empty or full? Anything left?

Sit for at least 3 minutes to give the poos time to come!

Have your child avoid straining or holding their breath.

Toileting Routine

At this stage your child's automatic response signal to pass bowel movements may not be working properly. He/she therefore needs to follow a toileting routine until the signal is working reliably again. This may take a few weeks or several months.

This signal is often triggered when the stomach is stretched after mealtimes, so 20-30 minutes after each main meal is an ideal time to schedule in a toilet time.

Encourage your child to sit on the toilet for 3 – 5 minutes (may need to be 2-3 mins for young children):

- After breakfast
- After lunch (this can be when they get home from school)
- After dinner

Try to build a toileting routine into your existing routines so that everyone remembers and you can get into a habit. For example: eat breakfast → clean teeth → pack bag → toilet time

During toilet time, talk with your child about how their body is feeling (empty, full, sore, rumby) and what that might mean. Teaching them to sit quietly and think about their body's signals is an important step in conquering poos! Talk with them about relaxing and concentrating on long, slow, deep breaths. Putting their hand on their tummy will show them how fast or slow they are breathing.

Alternate suggestion: Your child needs to concentrate on maintaining correct position, how their body feels and relaxing their bottom. However, sometimes a quiet conversation or story book may help them to relax and remain sitting for long enough to allow a poo to come out.

If rewards are being used, these should only be given after toilet time has finished, not while sitting.

How can I encourage my child to participate?

Although it is up to your child to develop effective toileting function, as a parent/carer you are able to assist by supervising and encouraging his/her efforts. You need to work together like a sports team. Your child is the “star player”, but your role is very important too, as the “head coach”! Tips to try:

- Remind your child they need to sit because their bowel doesn’t reliably tell them when they need to poo. If they sit regularly, they are more likely to do poos on the toilet than in their pants.
- Be consistently supportive but firm. Avoid compromises!
- Set a timer so toilet time has a predictable end
- Give your child some control over the situation through choice, for example: “after toilet time, would you like to play a game or read a story?” or “are you going to sit on the toilet for 2 minutes or 3 minutes?”
- If you are pleased with your child’s efforts, reward him/her straight away. Social praise with a big smile, clap hands and Hi 5 are always most helpful responses. Tangible rewards are not always essential. Instead you may wish to choose a more interactive reward with your child such as playing a game, going for a walk, or choosing (out of 2 given options) what the family has for dinner.
- Reward effort, not merely success.
- Try to maintain the routine regardless of where you happen to be at the time (e.g. visiting with someone or shopping).

Diary

Recording with a Bowel Diary

It is very important that you and your child keep a record of their progress. For younger children it is often better for you to keep a detailed bowel diary and allow your child a simple sticker chart. Keep the chart somewhere you can easily see it and add information and stickers! Many families keep it on the fridge or in the bathroom next to the toilet. Immediately after each toilet sit, help your child to record his/her sitting time, whether or not any poos came, and whether or not undies were clean or dirty. It is also useful to include information such as the date, when/how much medication was given that day, and any pain or other items of importance.

Example Poo Diary:

What to record?

Sit -

tick the box every time child sits on the toilet

Poo on toilet -

mark with a sticker or smile, and add the number of the Choose Your Poo chart that the poo most looks like

Soils -

mark with:

S = Skid/stain

C = Scrape (enough to scrape into toilet)

M = Motion (a whole poo - record what it looks like from Choose Your Poo)

TOILETING RECORD CHART for:

The purpose of this chart is to record your child’s daily bowel (poo) patterns.

Date	Time	Drink	Bowels/poos Pants or toilet	Choose your poos	Medication
					

When Soiling Happens

Soiling causes much frustration and annoyance, and you may be worried that your child will be (or is already being) teased. Soiling will reduce once a consistent toileting routine is established and the bowel is starting to recover. It may reduce quickly (in a few weeks) or slowly (over a few months). Soiling may even re-occur sometimes.

Often, children stay clean at school and soil as soon as they get home. This can be confusing and frustrating for parents. This is usually due to poor coordination of the anal muscles that control the opening and closing of the bottom to let poo out. There are two muscles around the anus. (one inside and one on the outside) and these should work together to keep the stool in the bottom until we reach a toilet.

However, with chronic constipation when the rectum is stretched the internal muscle does not work well. The child needs to consciously hold the outside muscle tightly shut. This takes a lot of effort and is not sustainable over the long term but may be just possible at school to avoid embarrassment. These children need to be prompted to sit on the toilet during lunchtime at school, and as soon as they get home.

Children do not soil on purpose (see 'Constipation and Soiling' section on page 4). Some children may hide their dirty pants to avoid upsetting their parents.

They may even act as if they don't care or that the soiling doesn't worry them. In reality, however, they do care but they just don't know how to deal with this overwhelming problem. Your acceptance of your child, patience and love is critical to help your child break through this vicious cycle.

What do I do if my child soils?

1. Do not scold them. Don't show your frustration. This is difficult at first. Take a deep breath to allow yourself time to respond without anger or frustration.
2. Have your child clean up as much as he/she can do independently.
3. Have a designated soak bucket that your child can put clothing in.
4. Have your child sit on the toilet for an extra toilet time in case there is more poo to come out.

Remember: Give your child as little attention as possible for the soiling. Do not 'fuss' over them, but remain matter-of-fact. Be aware that sometimes children repeat behaviours to get attention, even if it is negative attention.

Having your child involved in dealing with the consequences of soiling helps them feel more responsible and more likely to realise that the soiling is a problem.

Being Positive and Setting Goals

Ultimately it is up to your child to develop normal toileting behaviour, however your support and consistent approach is essential to help them along the way. Remember that a positive attitude will 'rub off' on your child as you work toward conquering poos together! Setting goals helps to break down a seemingly large problem into manageable steps. Goals need to be achievable, and often start with simple changes: for example, drinking one bottle of water at school each day or taking soiled undies to a soaking bucket.

The big goal is taking charge of poos, but there are many small steps to achieve before that becomes a reality! Goals need to be set according to your child's current abilities, and with cooperation between you and your child.

Resources

These resources may be useful in providing you with more information about childhood constipation/ soiling. This list is for your information only and should not replace advice given to you by your child's doctor or health professional.

- Continence Foundation of Australia has plenty of resources on childhood continence that can be downloaded or ordered free of charge

National Continence Helpline – Free call 1800 33 00 66

Website: www.continence.org.au

- The National Public Toilet Map is an initiative of the Federal Government's National Continence Management Strategy, and provides the location of 14 000 public toilets in Australia. You can plan trips and locate toilets in your local area.

Website <https://toiletmap.gov.au>

- Bladder and Bowel UK: <http://www.bladderandboweluk.co.uk>
- Australian parenting website – Raising Children Network: <http://raisingchildren.net.au>
- Another useful UK-based parent website – ERIC: <https://eric.org.uk/>
- Eat for Health (Australia) <https://www.eatforhealth.gov.au/nutrient-reference-values/nutrients/water>
- Children's Hospital Colorado – Educational video produced in 2013: *"The Poo in You – Constipation and Encopresis"* https://m.youtube.com/watch?v=SgBj7Mc_4sc
- An additional education video is: *"Constipation in Children: Understanding and Treating This Common Problem"* developed by the Paediatric Gastroenterology Unit at the Primary Children's Hospital in Utah: <https://www.youtube.com/watch?v=pNagQup0Upg>

- Children's Story books:
 - *"Softy the Poop - Helping Families Talk About Poop"*
by Thomas R. Duhamel 2015
 - *"Come Out Mister Poop"*
by Janelle McGuinness 2017
 - *"So You Think You Can Wipe"* by
Amber Hope 2015
 - *"The Incredible Poop Party"*
by Zoe Panto 2019
 - *"From Chewing to Pooing: A Food's
Journey Through Your Body to the
Potty"* by Lauren Gehringer with Dr
Natalie Gehringer 2021
 - *"Konstipated Koala: Dealing with
Constipation in Kids"*
by Dr Abraham Thomas 2022
 - *"How Do You Know You Need to
Go?"* by Wendy Hayden 2020

References

1. Queensland Health,
*Managing Chronic Constipation and
Soiling in Children: Conquering Poos
– A Guide for Parents*, 2011
2. Royal Children's Hospital, Melbourne,
Australia,
*Clinical Practice Guideline on
Constipation*,
[last revised March 2020]
Available from: https://www.rch.org.au/clinicalguide/guideline_index/Constipation
3. South Australian Paediatric Clinical
Practice Guidelines,
Constipation in Children
[Updated 2018]
Available from: <https://www.sahealth.sa.gov.au/wps/wcm/connect/public+content+sa+health+internet/clinical+resources/clinical+topics/pædiatric+clinical+guidelines>

Appendix A

How much fibre is in foods?

The following shows how many grams (g) of fibre are in foods that children and toddlers can include in their diet. High fibre foods are foods that are closer to the ground. This is because they have had less processing in food factories.

Fruit		
	Serve	Fibre (grams)
Pear with skin	1 medium	5
Apple with skin / orange	1 medium	3.5
Banana	1 medium	3
Strawberries	½ punnet	2
Sultanas	1 tablespoon	2
Prunes	6	4.7
Vegetables (cooked)		
Peas	½ cup	4.5
Potato (no skin)	1 medium	2
Broccoli	½ cup	3
Carrot/ Sweetcorn	½ cup	2.5
Cauliflower/ Pumpkin	½ cup	2
Cereals		
Sultana Bran®	1 cup	6.5
Weet-Bix®	2 biscuits	3.5
Porridge	1 cup	3.5
All Bran®	½ cup	12.6
Special K®	1 cup	2.4
Legumes		
Baked beans	½ cup	6
Red kidney beans	½ cup	6.5

Appendix B

Fibre for children and toddlers

Below are ideas on how toddlers and children can consume the amount of fibre they need.

Good sources of dietary fibre	
Bread	• Choose wholemeal and multigrain breads. High fibre white breads have less fibre than wholemeal bread, but are still a reasonable choice. • Try making sandwiches with one slice of wholemeal bread and one slice of white bread. • Choose wholemeal muffins and crumpets over ordinary varieties. • Wholemeal pita, pizza crusts and Lebanese breads. • Fruit/raisin loaf for a snack.
Cereals	• Choose wholegrain varieties (e.g. Weet-Bix®, VitaBrits®, Sultana Bran®, Mini Wheats®, Weet-Bix Bites®, muesli) • Porridge made with rolled oats. • Add a spoonful of wheat bran, oat bran or a bran mix to breakfast cereal. • Add dried fruit to breakfast cereals. • Try fruit balls made with crushed Weet-Bix® and diced dried fruit (see recipe).
Other bread and cereal products	• Choose wholemeal cracker biscuits (e.g. wholemeal Saladas® or wholemeal Premiums®) for a snack. • Air popped popcorn as a snack. • Swap white rice and pasta for brown rice and pasta. • Try baking cakes, biscuits or scones with a mixture of ½ wholemeal and ½ ordinary flour. • Use wholemeal breadcrumbs or oats in rissoles and burgers or for toppings on mornays and casseroles. • Add almond meal or LSA (linseed sunflower and almond meal) into cereal, or bread/cake mixes.

Fruit	• Encourage whole fruit rather than canned fruit or fruit juices. Whole fruit contains much more fibre in the skin, flesh and seeds. • Add dried fruit to cakes and biscuits. • Cut up fruit for a platter after school. • Freeze wedges of fruit on paddle pop sticks for an icy fruit snack. • Try frozen grapes for a change. Or make fruit into funny faces. • Offer freeze dried apples for a snack.
Vegetables	• Eat a wide variety of fresh vegetables. • Where possible, leave the skin (washed) on potatoes, zucchini, carrots and other vegetables. • Add grated or chopped vegetables to hamburger patties, pasta sauces (e.g. spaghetti bolognese), tacos, pizza. • Dip carrots, capsicum or celery in peanut butter or chick pea dip (hommus). • Offer salads as an alternative to cooked vegetables. • Puree mixed vegetables and use as a base for soups and sauces (such as spaghetti bolognese) • Try oven based veggie chips such as carrot or sweet potato. • Make the most of tinned or frozen vegetables as they are also very nutritious.
Dried peas, beans and lentils	• Try baked beans on toast for a fast healthy meal. • Add a can of red kidney beans to taco mix for a Mexican flavour. You can use them together with minced meat in dishes such as chilli con carne and spaghetti bolognese. • Try lentil burgers as an alternative to meat patties. • Buy bean sprouts at the market or 'sprout' your own and add these to salads or stir fries. • Try hommus on wholemeal dry biscuits for an after school snack.

Bran	It may not be necessary to add bran to your child's diet as enough fibre can usually be obtained from wholegrain breads and cereals, fresh fruit and vegetables. • If required, wheat, oat and rice bran and psyllium husk can be used to: • Add to breakfast cereals, stewed fruit and yoghurt. • Mix into meat patties, mince, casseroles and thick soups. • Add to biscuits, muffins, cakes and pancakes.
Nuts and seeds	Nuts and seeds (including coconut) are high in fibre. However, these foods are not suitable for younger children because they are a choking risk. Although they are healthy, nuts and seeds are also high in fat and should only be eaten in small quantities if your child has a weight problem. For younger children: • Nut pastes that are smooth can be used e.g. smooth peanut butter • Use almond meal or LSA (linseed, sunflower and almond mix) in cereal, smoothies, yoghurt or homemade breads, cakes and muffins For older children: • Try a handful of nuts and dried fruit as a snack. • Try muesli for a breakfast cereal or sprinkle on other cereals and on ice cream and fruit. • Add nuts and dried fruit to cakes and biscuits. Try spreading peanut butter on wholemeal bread for an after school snack.

Notes:

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

.....

For more information

Pædiatrics
Southern Adelaide Local Health Network
Flinders Medical Centre
Bedford Park SA 5042
Telephone: 08 8204 4910



This document has been reviewed
and endorsed by consumers in.



Interpreter



<https://creativecommons.org>

© Department for Health and Wellbeing,
Government of South Australia.
All rights reserved.
Version 2. Reviewed June 2024.



Government
of South Australia

Health
Southern Adelaide
Local Health Network